title: "Falls Prevention Policy" effectiveDate: "" reviewDate: "" version: "1.0"

Falls Prevention Policy

Policy Title: Falls Prevention

Effective Date: [Insert Date]

Review Date: [Insert Date – usually annually]

Version: 1.0

Applies to: All residents, staff, agency workers, contractors, and volunteers at [INSERT CARE HOME NAME]

1. Policy Statement

At [INSERT CARE HOME NAME], we are committed to reducing the risk of falls and ensuring the safety and wellbeing of all residents. Falls are a significant cause of injury, loss of independence, and reduced quality of life, and prevention is a key aspect of person-centred care.

We aim to:

- Identify residents at risk of falls through thorough assessment and monitoring.
- Implement proactive strategies to reduce the risk of falls and fall-related injuries.
- Ensure compliance with Health and Social Care Act 2008 (Regulated Activities) Regulations 2014 and CQC Fundamental Standards.
- Promote independence and dignity while balancing safety and risk management.

2. Purpose

This policy sets out how [INSERT CARE HOME NAME]:

- Assesses fall risk for all residents.
- Implements effective falls prevention measures.
- Responds to and manages incidents of falls.
- Monitors outcomes and promotes continuous improvement.

3. Scope

This policy applies to:

- Residents – all individuals receiving care within the home.
- Staff – all care and support staff responsible for resident wellbeing.
- Environment – all indoor and outdoor areas within the home.

4. Roles and Responsibilities

Registered Manager:

- Ensures falls prevention strategies are in place and monitored.
- Provides staff with training and resources.
- Reviews and analyses falls data to identify trends and improvements.

Nurses / Senior Care Staff:

- Complete falls risk assessments on admission and regularly thereafter.
- Develop and implement falls prevention care plans.
- Review and update plans following any fall or change in resident condition.

All Staff:

- Follow residents' individual care plans.
- Monitor residents for changes in mobility, balance, or behaviour.
- Report and document any falls or near-misses immediately.

5. Falls Risk Assessment

- All residents will have a Falls Risk Assessment completed on admission and reviewed:
    - At regular intervals (minimum annually).
    - After any fall.
    - Following changes in health or medication.
- Assessments will cover:
    - Mobility and balance.
    - Medication (particularly sedatives, antihypertensives, or polypharmacy).
    - Vision and hearing.
    - Footwear and clothing.
    - Environment (flooring, lighting, hazards).
    - Cognitive status and continence.

6. Falls Prevention Measures

- Environmental Safety – good lighting, clear walkways, non-slip flooring, handrails, call bells within reach.
- Mobility Support – use of walking aids, physiotherapy referrals, exercise programmes to maintain strength and balance.
- Medication Review – regular review by healthcare professionals to minimise fall-related side effects.
- Footwear & Equipment – safe, well-fitting shoes and properly maintained equipment.
- Staff Support – prompt response to call bells, appropriate supervision, and resident observation.
- Resident Engagement – encourage independence while supporting safe choices.

7. Responding to Falls

- Staff must provide immediate assistance while ensuring their own safety.
- First Aid provided and medical help sought if required.
- Resident must not be moved until safe to do so (use of lifting aids as appropriate).
- Incident recorded in resident notes and accident log.
- Family/next of kin informed.
- Post-fall review conducted to identify causes and update care plan.

8. Monitoring and Review

- All falls and near-misses will be documented and reviewed by management.
- Monthly falls audits carried out to identify trends and high-risk residents.
- Findings used to update care plans and prevention strategies.
- Lessons learned shared with staff through supervision and training.

9. Training

- All staff receive training in:
    - Falls awareness and prevention.
    - Safe use of equipment and mobility aids.
    - Emergency response to falls.
- Refresher training provided at least annually.

10. References

- Health and Social Care Act 2008 (Regulated Activities) Regulations 2014
- CQC Fundamental Standards – Regulation 12 (Safe Care and Treatment)
- NICE Guideline NG5: Falls in older people – assessment and prevention
- HSE Guidance on Slips, Trips and Falls

11. Related Policies

- Risk Assessment Policy
- Health & Safety Policy
- Safeguarding Adults Policy
- Incident Reporting Policy
- Training & Development / Supervision Policy
- Medication Management Policy

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